

Class 2 – Elbow Fracture: Patient Care Guide

A clear, patient-friendly guide for people who have an elbow fracture — a break in one of the bones that form the elbow joint (the lower humerus, the olecranon of the ulna, or the radial head). It explains causes, symptoms, diagnosis, treatment, healing, exercises, warning signs, and common questions.

Medical disclaimer: This document is an educational reference created for a patient-support knowledge base and for use in a retrieval-augmented (RAG) AI assistant. It is NOT a substitute for professional medical advice, diagnosis, or treatment. Healing times, medicines, and exercises differ for every person. Always follow the instructions of your own doctor, orthopaedic surgeon, or physiotherapist. In an emergency, contact local emergency services immediately.

1. Introduction

An elbow fracture is a break in one of the three bones that meet at the elbow joint. The elbow is a hinge joint that also allows the forearm to rotate. A break here is also called a broken elbow.

Which bones are involved: the lower end of the humerus (upper arm bone), the olecranon (the bony tip of the ulna that forms the point of the elbow), and the radial head (the top of the radius). Any one or a combination can break.

Common causes: falling onto the elbow or an outstretched hand, direct blows, sports injuries, and road accidents. How common it is: elbow fractures are common in both children (especially supracondylar fractures of the humerus) and adults, and account for a large share of arm injuries.

Who is at higher risk: children (elbow fractures are among the most common childhood fractures), older adults with osteoporosis, athletes, and people in high-fall-risk activities. Severity ranges from a small undisplaced crack (mild) to a shattered, displaced, or open joint fracture (severe).

Types of Fractures Affecting This Bone

A elbow fracture can happen in several patterns. Common fracture types include:

- Hairline fracture (a very thin, small crack in the bone).
- Stress fracture (a tiny crack caused by repeated force or overuse).
- Greenstick fracture (the bone bends and cracks on one side; common in children).
- Closed fracture (the bone breaks but the skin stays intact).
- Open (compound) fracture (the broken bone breaks through the skin).
- Comminuted fracture (the bone breaks into three or more pieces).
- Spiral fracture (a break that twists around the bone, often from a twisting injury).
- Impacted fracture (the broken ends are driven into each other).
- Transverse fracture (a straight break across the bone).
- Oblique fracture (a slanted or angled break across the bone).

Common elbow patterns include olecranon fractures (the elbow tip), radial head fractures (top of the radius), and supracondylar fractures (just above the elbow in children).

2. Anatomy

The elbow is where three bones meet, wrapped by muscles, ligaments, nerves, and vessels.

- Bones involved: the distal humerus, the olecranon (ulna), and the radial head (radius). Together they allow bending, straightening, and forearm rotation.
- Nearby muscles: the biceps and brachialis bend the elbow; the triceps straightens it; forearm muscles attach around the joint.
- Tendons: the triceps tendon attaches to the olecranon and the biceps tendon near the radius; injury affects the ability to bend or straighten the elbow.
- Ligaments: the collateral ligaments on each side stabilise the joint against side-to-side movement.
- Blood vessels: the brachial artery passes in front of the elbow to supply the forearm and hand; injury can threaten blood flow.
- Nerves: the ulnar nerve (the 'funny bone'), median, and radial nerves cross the elbow and control hand movement and feeling; they can be stretched or pressed in elbow fractures.

Because the elbow is a joint, breaks that involve the joint surface can lead to stiffness and arthritis if not aligned well. Nerve injury (especially the ulnar nerve) can cause hand numbness, so doctors check hand function carefully.

3. Common Causes

Most fractures happen when a force is stronger than the bone can handle. The main causes are:

- Falls: slipping, tripping, or falling onto an outstretched hand or arm — the most common cause overall.
- Road traffic accidents: high-speed impacts from car, motorbike, or cycle crashes.
- Sports injuries: contact sports, skating, cycling, gymnastics, and falls during play.
- Workplace accidents: falls from height, machinery injuries, or heavy objects.
- Direct trauma: a hard blow or hit directly on the bone.
- Twisting injuries: the limb turns while the hand or body stays fixed.
- Heavy object injuries: something heavy falling onto the limb.
- Osteoporosis: weak, thin bones that break easily, especially in older adults.
- Bone tumors: growths that weaken the bone (pathological fracture).
- Repetitive stress: repeated overuse leading to stress fractures over time.

For the elbow specifically, a fall directly onto the point of the elbow or onto an outstretched hand is the leading cause.

4. Symptoms

Common signs and symptoms of an elbow fracture include:

- Sudden, sharp pain in the elbow.
- Swelling and bruising around the joint.
- A deformed or out-of-shape elbow.
- Inability to straighten or bend the elbow.
- Difficulty turning the palm up or down.

- Numbness or tingling in the hand or fingers (possible nerve involvement).
- Weak grip or trouble using the hand.
- A popping or cracking feeling at the time of injury.
- In an open fracture, bone may be visible through a wound.

Emergency warning signs: a cold, pale, or blue hand, no pulse at the wrist, spreading numbness, severe deformity, bone through the skin, or rapidly increasing pain. Get emergency care at once — the brachial artery and nerves can be at risk.

5. Diagnosis

Doctors confirm a fracture using a physical check and imaging tests. Each test gives different information.

Physical Examination and Medical History

The doctor asks how the injury happened and checks for pain, swelling, bruising, deformity, tenderness, movement, pulse, skin colour, and sensation. Telling the doctor about past fractures, osteoporosis, medicines, and other illnesses helps plan the right treatment.

Imaging Tests

- X-ray: the first and main test. It shows most fractures, the exact location, and whether the bone has moved out of place (displacement).
- CT scan: gives detailed 3-D images. Used for complex breaks, joint involvement, or when the X-ray is unclear, to plan surgery.
- MRI: shows soft tissues, bone bruising, and hairline or stress fractures that may not appear on an X-ray. Also checks nearby ligaments, tendons, and nerves.
- Ultrasound: sometimes used, especially in children, to look at bone surfaces and soft-tissue swelling without radiation.
- Bone scan: can detect very small stress fractures or hidden fractures that other tests miss, by showing areas of increased bone activity.

For an elbow fracture, X-rays from two angles are standard. A CT scan is often used because joint-surface (intra-articular) fractures are common and need precise assessment before surgery. In children, comparison X-rays of the other elbow may help because growth plates can look like fractures.

6. Treatment Options

Treatment depends on the type of break, how far the bone has moved, and your age and health. It falls into non-surgical and surgical options.

Non-Surgical Treatment

Undisplaced or stable elbow fractures often heal without surgery.

- Cast or splint: holds the elbow, usually bent at about 90 degrees, while it heals.
- Brace: a hinged brace may allow controlled, protected movement in some fractures.
- Sling: supports the arm and keeps the elbow at rest.

- Ice therapy: reduces pain and swelling in the first days.
- Elevation: keeping the arm raised lowers swelling.
- Pain medications: paracetamol for pain relief.
- Anti-inflammatory medicines: such as ibuprofen, if approved by your doctor.

When recommended: non-surgical care suits stable fractures where the bone is in a good position. Advantages: no surgery, lower infection risk, and no anaesthesia. Limitations: needs a well-set position, the cast can cause stiffness, and it may not hold badly displaced or unstable fractures.

Surgical Treatment

Surgery is used when the bone is badly out of place, broken into many pieces, involves a joint, is an open (compound) fracture, or fails to heal with a cast. The aim is to put the pieces back in the correct position and hold them still while they heal.

- ORIF (Open Reduction Internal Fixation): the surgeon opens the skin, sets the bone in the right position (reduction), and fixes it with metal implants.
- Plates: flat metal supports screwed along the bone to hold the pieces together.
- Screws: hold fragments together, alone or with a plate.
- Pins (K-wires): thin wires that hold small pieces, often used in fingers and in children; usually removed later.
- Rods (intramedullary nails): metal rods placed inside the hollow centre of a long bone to support it.
- External fixator: a frame outside the body connected to the bone by pins; used for severe, open, or badly contaminated fractures.
- Bone graft: extra bone (from your own body or a donor) added to help healing when there is bone loss or slow healing.

Indications (when surgery is needed): displaced olecranon fractures, comminuted or joint-surface fractures, unstable radial head fractures, open fractures, and most displaced supracondylar fractures in children (often fixed with pins). Olecranon fractures are commonly fixed with a tension-band wire or a plate.

Recovery Expectations and Risks After Surgery

- Recovery: a protective cast, splint, or sling is often used after surgery, followed by physiotherapy. Implants usually stay in place unless they cause problems.
- Risks: infection, bleeding, nerve or blood vessel injury, implant irritation or loosening, stiffness, blood clots, and reaction to anaesthesia.
- Most people regain good function, but full recovery takes weeks to months and depends on the injury and following rehabilitation.

7. Healing Timeline

Healing time varies by age, general health, fracture type, and how well instructions are followed. The table gives typical ranges.

Group	Typical healing / recovery
Children	Often 3–6 weeks to unite; pinned supracondylar fractures heal quickly but need careful follow-up.
Adults	About 6–12 weeks for bone union; joint fractures may need longer rehab.
Older adults	8–12 weeks or more; stiffness is a common challenge.
Athletes	Union in 6–12 weeks; full return to sport often 3–4 months.

Stages of healing: initial healing (inflammation and soft callus) in the first weeks, bone union (hard callus bridges the break), and functional recovery (regaining strength and movement). Regaining full straightening (extension) of the elbow is often the slowest part and needs steady physiotherapy.

- Return to work: desk work in 1–3 weeks; heavy manual work often 10–12 weeks or after clearance.
- Return to sports: non-contact around 8–12 weeks; contact/throwing sports usually 3–4 months.
- Full recovery: 3–6 months; some stiffness can take up to a year to fully settle.

Factors that delay healing: smoking, alcohol, poor nutrition, diabetes, osteoporosis, infection, poor blood supply, older age, certain medicines (such as steroids), and not resting the injured limb.

8. Recovery Stages

Knowing what to expect at each stage helps you feel reassured. Your own timeline may differ slightly.

Stage	What usually happens
Week 1	Pain and swelling peak; arm rested in splint/sling; move fingers and shoulder.
Week 2	Swelling easing; keep hand and shoulder moving to prevent stiffness.
Week 3–4	Bone knitting; gentle, protected elbow movement may begin if advised.
Week 5–8	Bone uniting; splint often removed; active physiotherapy for bending/straightening.
Month 3	Bone usually united; working on full range and strength.
Month 6	Near-full movement and strength for most; heavy activity resumed.

9. Daily Care Instructions

- Keep the cast or splint dry: cover it while bathing; moisture weakens it and irritates skin.
- Sleeping position: keep the arm supported and slightly raised on a pillow; avoid lying on the injured elbow.
- Bathing: use a waterproof cover or sponge-bathe; keep the elbow still if in a splint.
- Clothing: wear loose or button clothing; dress the injured arm first.
- Walking: normal walking is fine; keep the arm in the sling for support.
- Driving: avoid driving until the elbow bends enough and you can control the car safely — ask your doctor.
- Returning to office: light desk work is often possible within 1–3 weeks.

- Returning to school: children can attend once comfortable, avoiding sport and rough play.
- Household activities: avoid pushing, pulling, and lifting with the injured arm.

10. Pain Management

Some pain is normal after a fracture, especially in the first days. These safe, simple methods help control it.

- Ice packs: wrap ice in a cloth and apply for 15–20 minutes several times a day to reduce pain and swelling (never put ice directly on the skin).
- Elevation: keep the injured limb raised above the level of the heart to reduce throbbing and swelling.
- Rest: avoid using the injured limb for gripping, lifting, or bearing weight.
- Painkillers: paracetamol (acetaminophen) is commonly used. Anti-inflammatory medicines such as ibuprofen may be advised. Take only the dose your doctor approves.
- Safe positioning: support the limb on a pillow and avoid letting it hang down.
- Relaxation techniques: slow breathing, distraction, and gentle music can reduce the sensation of pain.

When pain is abnormal: pain that suddenly becomes severe, is not relieved by medicine, comes with numbness, a cold or blue hand, or a very tight cast may signal a serious problem such as compartment syndrome or poor blood supply. Seek medical help urgently.

11. Diet for Bone Healing

Good nutrition speeds up bone healing. The nutrients below all play a role. Try to eat a balanced diet that includes them every day, and follow any special diet advice from your doctor or dietitian.

Key Nutrients for Bone Healing

Nutrient	Why it helps & daily intake	Vegetarian sources	Non-vegetarian sources
Calcium	Main mineral that builds and repairs bone. About 1000–1200 mg/day for adults.	Milk, curd, paneer, cheese, ragi, tofu, green leafy vegetables, sesame seeds	Small fish eaten with bones (sardines), dairy
Vitamin D	Helps the body absorb calcium. About 600–800 IU/day (more if deficient).	Sunlight, fortified milk and cereals, mushrooms	Egg yolk, fatty fish (salmon, mackerel), fish liver oil
Protein	Builds the collagen framework of bone. About 1.0–1.5 g per kg body weight.	Dal, beans, chickpeas, rajma, soya, paneer, tofu, nuts	Eggs, chicken, fish, lean meat
Vitamin C	Needed to make collagen for bone repair. About 65–90 mg/day.	Oranges, lemon, amla, guava, capsicum, tomato, broccoli	(Mainly from plant foods)
Vitamin K	Helps bind calcium into bone. About 90–120 mcg/day.	Spinach, kale, broccoli, cabbage, green leafy vegetables	Eggs, some dairy

Nutrient	Why it helps & daily intake	Vegetarian sources	Non-vegetarian sources
Magnesium	Supports bone structure and vitamin D use. About 310–420 mg/day.	Nuts, seeds, whole grains, bananas, dark leafy greens	Fish
Phosphorus	Works with calcium to harden bone. About 700 mg/day.	Dairy, beans, lentils, whole grains, nuts	Meat, fish, poultry, eggs
Zinc	Helps bone cells grow and repair. About 8–11 mg/day.	Whole grains, legumes, seeds, nuts	Meat, shellfish, eggs
Omega-3 fats	Reduce inflammation and support healing.	Flaxseed, chia seeds, walnuts, soybean oil	Fatty fish (salmon, mackerel, sardines)
Collagen	Forms the flexible base of bone. Supported by protein + vitamin C.	Body makes it from protein-rich plant foods + vitamin C	Bone broth, chicken, fish, eggs

Drink enough water and, if your doctor recommends, take calcium and vitamin D supplements. Do not take high-dose supplements on your own without medical advice.

12. Foods to Avoid

Some foods and habits slow bone healing by reducing calcium absorption, increasing inflammation, or harming blood supply. Limit or avoid the following while your fracture heals.

- Alcohol: slows new bone formation and reduces calcium and vitamin D use. Best avoided completely during healing.
- Smoking and tobacco: reduce blood flow to the bone and are a leading cause of delayed healing and nonunion. Stopping greatly improves healing.
- Sugary foods: too much sugar can increase inflammation and displace healthy foods.
- Soft drinks and colas: high phosphoric acid and sugar may reduce calcium balance.
- Excess caffeine: large amounts of coffee, tea, or energy drinks can reduce calcium absorption; keep intake moderate.
- Processed and packaged food: often high in salt and low in nutrients.
- High sodium (salt) intake: makes the body lose more calcium in urine.
- Junk and deep-fried food: high in unhealthy fat and low in bone-building nutrients.

In short, a fresh, home-cooked, balanced diet with less salt, sugar, alcohol, and no smoking gives your bone the best chance to heal quickly.

13. Physical Therapy and Rehabilitation Exercises

Physiotherapy restores movement, strength, and function. Start only when your doctor allows, and stop any exercise that causes sharp or increasing pain. Below are typical stages — your therapist will tailor them to you.

Early Stage

- Finger, wrist, and shoulder movements — purpose: prevent stiffness while the elbow rests. Repetitions: gentle, 10 each, several times a day. Precaution: keep the elbow still if instructed.
- Gentle hand squeezing — purpose: maintain grip and reduce swelling.

Middle Stage

- Active elbow bending and straightening (when allowed) — purpose: restore the hinge movement. Repetitions: 10 slow reps, 2–3 times daily. Precaution: no forcing into pain.
- Forearm rotation (palm up/down) — purpose: regain rotation. Repetitions: 10 slow turns.
- Assisted stretches to regain full straightening — purpose: reduce stiffness.

Advanced Stage

- Light resistance for biceps and triceps — purpose: rebuild strength. Repetitions: 10–15, 2–3 sets. Precaution: start with very light weight.
- Grip and forearm strengthening with putty or bands — purpose: restore hand strength.
- Sport-specific and lifting progression — purpose: safe return to full activity. Precaution: increase slowly; stop if pain or swelling returns.

General exercise precautions: move slowly, never push through sharp pain, stop if you feel numbness or dizziness, and follow the exact repetitions your therapist advises. When to stop: stop and call your therapist if pain worsens, swelling increases, or movement suddenly decreases.

14. Activities to Avoid During Healing

Avoiding certain activities protects the healing bone and implants.

Activity	How long to avoid (typical)
Leaning or pushing up on the elbow	Until healed — often 6–8 weeks
Heavy lifting / carrying with that arm	About 10–12 weeks or until cleared
Throwing sports and racket sports	About 3–4 months
Push-ups, pull-ups, weight training	About 3 months or until cleared
Contact sports (football, martial arts)	About 3–4 months
Driving with a stiff or splinted elbow	Until safe control and doctor clearance

15. Possible Complications

Most fractures heal well, but knowing the possible problems helps you spot them early. Report any of these to your doctor.

- Delayed union: the bone heals more slowly than expected. Prevented by good nutrition, not smoking, and following restrictions.

- Nonunion: the bone fails to join. Signs are ongoing pain and movement at the break site; may need surgery or a bone graft.
- Malunion: the bone heals in the wrong position, causing deformity or reduced function; sometimes needs correction.
- Infection: mainly with open fractures or after surgery. Signs are fever, redness, warmth, swelling, pus, or bad smell.
- Stiffness: nearby joints become stiff from being kept still; prevented by physiotherapy and gentle movement as advised.
- Nerve injury: causes numbness, tingling, or weakness; often improves but should be checked.
- Blood vessel injury: rare but serious; causes a cold, pale, or blue limb and needs emergency care.
- Chronic (long-lasting) pain: pain that continues after healing; managed with therapy and pain specialists.
- Arthritis: joint fractures can lead to wear-and-tear arthritis years later.
- Reduced mobility: lasting loss of full movement, reduced by good rehabilitation.
- Complex Regional Pain Syndrome (CRPS): an uncommon condition with severe pain, swelling, skin colour and temperature changes; early treatment gives the best result.

16. Red Flag Symptoms — Go to Hospital Immediately

Get emergency medical help right away if you notice any of the following. These can mean the blood supply, nerves, or the healing is in danger.

- Severe or rapidly increasing swelling.
- Fingers (or toes) turning blue, grey, or very pale.
- The hand or limb feels cold to touch.
- Numbness or loss of feeling that does not go away.
- Fever or chills (a sign of infection).
- Bleeding that will not stop.
- Severe pain that is not relieved by rest, elevation, or prescribed medicine.
- The cast becomes too tight or the limb feels squeezed.
- The cast breaks, cracks, or becomes loose.
- Pus, discharge, or fluid coming from under the cast or a wound.
- A bad smell coming from the cast or wound.
- Bone is visible through the skin (open fracture).
- Difficulty breathing, chest pain, or coughing after the injury (a possible blood clot or fat embolism — a rare but serious emergency).

Compartment syndrome warning: severe deep pain, tightness, numbness, and pain when the fingers are gently stretched can signal dangerous pressure build-up. This is an emergency — do not wait.

17. Follow-Up Schedule

Regular follow-up lets the doctor check that the bone is healing in the right position. A typical schedule is below.

Visit	What the doctor usually checks
Week 1	Check swelling, splint fit, blood flow, nerve function, and position on X-ray.
Week 2	Confirm the bone/pins have not shifted; check the wound if operated.
Week 4	X-ray for early healing; often begin gentle protected movement.
Week 6	Check union; remove pins in children if used; progress physiotherapy.
Week 12	Confirm full union; assess range of motion, strength, and plan return to activity.

18. Returning to Normal Life

These are general guides for when people usually return to activities. Always get your doctor's clearance first.

Activity	Typical time to return
School	A few days to 1–2 weeks (no sport/rough play)
Office (desk job)	1–3 weeks
Driving	After good elbow movement and control — often 6–9 weeks
Gym (upper body)	About 3 months, building up gradually
Swimming	After splint off and skin/wound healed — about 8–12 weeks
Cycling	Light cycling around 8–12 weeks (elbow must bend and be pain-free)
Running	Often 6–8 weeks once the arm is stable
Cricket / Football	About 3–4 months with clearance
Weight lifting	About 3–4 months, increasing load slowly

19. Prevention of Future Fractures

After one fracture, it is worth protecting your bones for the future. These steps lower the chance of breaking a bone again.

- Bone-strengthening exercises: regular resistance and muscle-strengthening activity keeps bones dense.
- Weight-bearing exercise: walking, jogging, dancing, and stair climbing build bone strength.
- Balance training: yoga, tai chi, and balance drills reduce falls, especially in older adults.
- Vitamin D: from sunlight, diet, or supplements as advised, to help absorb calcium.
- Calcium intake: eat enough calcium-rich foods every day.
- Protective equipment: wrist guards, elbow pads, and helmets during sports and skating.
- Safe workplace habits: follow safety rules, use guards, and prevent falls from height.
- Sports safety: warm up, use correct technique, and wear the right gear.

- Fall prevention: good footwear, adequate lighting, and vision checks.
- Home safety tips: remove loose rugs, add grab bars, keep floors dry and clutter-free.
- Osteoporosis screening: a bone density (DEXA) scan for older adults and high-risk people, with treatment if bones are weak.

20. Frequently Asked Questions (FAQ)

Q1. Can I move my fingers?

Yes. Move your fingers, wrist, and shoulder gently while your elbow rests. This prevents stiffness and swelling. Keep the elbow itself still unless told otherwise.

Q2. Can I sleep on the injured arm?

No, avoid lying on the injured elbow. Support the arm on a pillow, slightly raised, for comfort and less swelling.

Q3. Can I remove the splint or cast?

No. Do not remove or adjust it yourself. It keeps the elbow in the right position until the doctor says it can come off.

Q4. When can I drive?

Only after your elbow bends enough and you can control the car safely — often around 6–9 weeks. Confirm with your doctor.

Q5. Can I exercise?

Gentle finger, wrist, shoulder, and lower-body exercise is usually fine early. Wait for advice before elbow and upper-body strengthening.

Q6. Can I lift weights?

Not until the bone heals and you are cleared — usually about 3–4 months, starting light.

Q7. Is swelling normal?

Yes, in the first days to weeks. Keep the arm raised and use ice. Report sudden, severe, or increasing swelling urgently.

Q8. Is itching inside the cast normal?

Yes, mild itching is common. Never insert objects to scratch. Cool air can relieve it.

Q9. Can I travel?

Short trips are fine once pain is controlled and the arm is supported. Ask before long journeys.

Q10. Can I fly?

Usually yes, but tell your doctor, especially soon after a new cast, because swelling can increase at altitude.

Q11. Can I get the cast wet?

No. Keep a standard cast dry with a waterproof cover during bathing to protect the skin and cast.

Q12. How long should I wear the sling?

Usually 1–3 weeks for comfort and support, or as your doctor advises.

Q13. When should I see a doctor urgently?

For a cold or blue hand, no wrist pulse, spreading numbness, severe pain, a very tight cast, fever, or discharge — seek care now.

Q14. Why is my elbow so stiff?

Elbows become stiff easily after injury. This is common and improves with regular physiotherapy; do not force painful movement.

Q15. Will I get full straightening back?

Most people regain most or all movement, but full straightening can be the slowest to return and may take several months.

Q16. Will I need surgery?

Not always. Undisplaced fractures heal in a splint. Displaced olecranon, joint, or unstable fractures often need surgery.

Q17. Do the wires or plates need removal?

Plates usually stay for life. Tension-band wires that irritate the skin, and children's pins, are often removed later.

Q18. My hand feels numb — is that serious?

Numbness can mean nerve irritation, often the ulnar nerve. Tell your doctor. It frequently improves but must be checked.

Q19. Can my child's elbow fracture affect growth?

Most heal well, but because growth plates are near the elbow, children need careful follow-up to ensure normal growth.

Q20. Is some pain normal weeks later?

Mild aching during healing and physiotherapy is normal. Sharp or worsening pain should be reviewed.

Q21. What foods help healing?

Calcium, vitamin D, protein, and vitamin C support bone repair. Avoid smoking and alcohol, which slow healing.

Q22. Can I go to work?

Desk work is often possible in 1–3 weeks. Heavy manual work usually needs 10–12 weeks or clearance.

Q23. Will my arm be weak after the splint comes off?

Yes, temporary weakness and stiffness are expected. Physiotherapy restores strength and movement.

Q24. Can I play sports again?

Yes, most people do. Non-contact around 8–12 weeks and contact/throwing sports around 3–4 months, after clearance.

Q25. How can I prevent another elbow fracture?

Strengthen bones and muscles, prevent falls, wear elbow protection in sport, and get osteoporosis screening if at risk.

Q26. Can I bend my elbow to eat or write?

Once your doctor allows gentle movement, light use like eating or writing is encouraged if it is not painful.

21. Metadata (for Retrieval)

Structured tags to help the AI assistant retrieve this document accurately.

Field	Value
Fracture Name	Elbow Fracture
Bones Involved	Distal humerus, Olecranon (ulna), Radial head (radius)
Body Region	Upper limb (elbow joint)
Common Causes	Falls onto elbow or outstretched hand, direct trauma, sports, road accidents, osteoporosis
Symptoms	Pain, swelling, deformity, unable to bend/straighten, numbness, weak grip
Treatment Options	Cast, splint, sling, hinged brace; ORIF, tension-band wire, plates, screws, pins, external fixator, bone graft
Healing Duration	Children 3–6 weeks; adults 6–12 weeks; full recovery 3–6 months
Rehabilitation Duration	8–16 weeks of physiotherapy; stiffness may take longer
Emergency Signs	Cold/blue hand, absent pulse, nerve loss, open bone, compartment syndrome
Keywords	elbow fracture, broken elbow, olecranon fracture, radial head fracture, supracondylar fracture, funny bone injury
Synonyms	Broken elbow, cracked elbow, elbow joint fracture
Related Injuries	Forearm fracture, humerus fracture, radial head dislocation, ulnar nerve injury